NCCP 00205 · clinician draft

Pegylated liposomal doxorubicin · every 4 weeks

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Pegylated liposomal doxorubicin · every 4 weeks · reviewed supportive care

Active medicines: Pegylated liposomal doxorubicin

Emetogenic risk: Low

Medicine	Dose / route	When
Ondansetron	8 mg orally once	Each treatment day only Review QT, constipation and hepatic dosing; reconcile any steroid already prescribed
Metoclopramide	10 mg orally up to three times daily as needed	Subsequent days: duration to be confirmed Assess contraindications, renal function and interactions

Additional phase-specific support

- No routine premedicines are specified. Low-risk antiemetic support is separate. Infusion reactions may require a different supervised plan for future doses.
- This is the pegylated liposomal formulation (for example Caelyx), not conventional doxorubicin or non-pegylated Myocet. These formulations and doses must not be interchanged.
- Assess G-CSF according to individual risk and the exact source; it is not automatically prescribed by this template.
- Low emetic risk with an individual prevention/rescue plan; no routine hypersensitivity premedication is usually required.
- Reduce heat and friction affecting palms/soles; report painful redness, peeling or mouth ulcers before the next dose.
- Review cardiac symptoms and lifetime anthracycline exposure; a liposomal formulation still needs cardiac surveillance.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00205 v8 · p4 · NCCP antiemetic guidance; reconcile with regimen and individual prescription

Sources checked 2026-10-25 · SACTCheck v0.84.21 · Regimen-level drafts; prescriber and oncology-pharmacy verification required